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Abstract

This study explores how socioeconomic factors influence suicide rates and access to mental health care in the United States. Despite advances in mental health treatment, suicide remains a leading cause of death, with persistent disparities in access to care among low-income and marginalised groups. Using a systematic literature review of three cross-sectional U.S. studies, this research identifies how poverty, income inequality, and structural barriers (such as cost, stigma, and provider shortages) contribute to mental health disparities and elevated suicide risk. Findings highlight the prevalence of mental disorders like depression, anxiety, and PTSD among vulnerable populations, particularly young adults, veterans, and ethnic minorities. The analysis emphasises the need for integrated policy solutions addressing economic inequality, healthcare accessibility, and culturally competent care. Recommendations include expanding mental health coverage, improving affordability, and embedding mental health considerations into broader social policies to reduce structural barriers and promote equitable mental health outcomes nationwide.

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How do socioeconomic factors influence suicide rates and access to mental health care in the U.S.?

Introduction

Background

Mental health is a state of well-being that allows individuals to manage life's challenges, recognise their strengths, learn effectively, perform well at work, and actively participate in their community (WHO, 2022). It is a fundamental aspect of overall health, supporting our ability to make decisions, form relationships, and influence the world (WHO, 2022). As a fundamental human right, mental health is essential for personal growth, community well-being, and socio-economic progress (WHO, 2022).

Suicide is now the 10th leading cause of death nationwide and the second leading cause among individuals aged 10 to 34. From 2000 to 2018, suicide rates increased by 36.7% (Tadmon & Bearman, 2023). In 2019 alone, an estimated 47,511 Americans died by suicide, with 1.4 million suicide attempts and over 12 million adults reporting serious suicidal thoughts (Tadmon & Bearman, 2023). The economic impact of suicide (factoring in medical costs and lost productivity) reached roughly $70 billion that year (Tadmon & Bearman, 2023).

Individuals with lower incomes, especially those below the poverty line, are at a higher risk of experiencing poor mental health and well-being (Kromydas et al., 2021). Similarly, unemployment is also linked to an increased likelihood of mental health challenges (Kromydas et al., 2021).

Problem Statement

Although evidence-based mental health treatments are more available, depression and suicide rates in the U.S. have not declined (Perry et al., 2022). Access remains limited due to persistent barriers such as stigma, cost, and logistical challenges. These issues

disproportionately affect certain groups, with notable disparities across age, gender, race, geography, veteran status, sexual orientation, and socioeconomic status.

Research Aim and Objectives

This report explores how socioeconomic factors (like income, poverty, education, employment, and healthcare access) affect suicide rates and mental healthcare availability across U.S. demographic groups, aiming to identify disparities and policy solutions.

This aim will be achieved through the following objectives:

- To determine the most common mental health disorders in the U.S. and the demographics most impacted by them.
- To analyse the link between income inequality, poverty, and suicide rates across U.S. demographic groups.
- To identify barriers to mental health care for low-income populations and their role in increasing suicide risk.

These objectives will be achieved with the help of the following research questions:

1. Which mental health problems are most common in the U.S., and who do they affect the most?
2. How do income inequality and poverty levels correlate with suicide rates across different U.S. demographic groups?
3. What barriers limit access to mental health care for low-income populations, and how do these barriers contribute to suicide risk?

Significance of the Study

This study highlights how socioeconomic disparities influence suicide risk and mental health in the U.S., offering insights for targeted interventions and policy reform. It adds to the broader discussion on social determinants of health by addressing structural barriers in mental healthcare.

Research Methodology

A systematic keyword-based search strategy was implemented to identify pertinent literature, followed by rigorous screening procedures to select studies for in-depth review.

Report Structure

Following this introductory chapter, the ethical statement will be presented, followed by an overview and strategy for the investigation. The report will present, discuss, and analyse the findings, concluding with a reflective conclusion.

Ethical Statement

To maintain academic integrity, all secondary data sources were properly referenced in line with Arden University GDPR and Harvard, with ethical standards. This study also followed key ethical principles:

All secondary data, whether physical or digital, was stored securely. Where necessary, permissions and approvals were obtained from relevant institutions, regulatory bodies, and administrative agencies to protect confidentiality and privacy, restricting access to authorised researchers only. Data was used responsibly to prevent any potential harm or discomfort. The research was conducted with transparency and integrity at every stage. Conclusions were derived from precise interpretations of the findings, ensuring no misleading or incorrect assertions were made.

Overview and Research Approach

A comprehensive literature review highlights the meanings and distinctions between theory, theoretical framework, and conceptual framework, which are critical for developing strong research (Kivunja, 2018). This process helps clarify the role of a theoretical framework, providing researchers with practical guidance on its application in academic studies (Randles & Finnegan, 2023).

Overview of Studies Included

This study conducted a systematic literature review using PubMed, Google Scholar, ResearchGate, ScienceDirect, and NIH. Three key papers were selected for their relevance to how socioeconomic factors affect suicide rates and mental healthcare access in the US.

Study	Country	Study Design	Sample Size	**Sex**	**Age**
(Hasin et al., 2018)	U.S.	Cross-Sectional	36,309 Adults	Female and Male	18-64 years
(Perry et al., 2022)	U.S.	Cross-Sectional	All 50 U.S. states + Washington D.C.	Female and Male	(15-24 years) (35-64 years) (65+ years)
(Coombs et al., 2021)	U.S.	Cross-Sectional	50,103 Adults	Female and Male	18-85+ years

Table 1 – Table analysis of the three chosen primary Research Papers.

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Impact of the most common mental health disorders in the U.S.

Mental health disorders significantly burden individuals and society. Major Depressive Disorder (MDD) affects 20.6% of adults' lifetime, with higher rates among women and young adults (Hasin et al., 2018). According to Santo et al. (2021), mental

health disorders include various conditions, such as anxiety disorders, mood disorders, and substance use disorders. In 2019, about 61.2 million U.S. adults (ages 18 and older) experienced a mental health disorder in the previous year (Santo et al., 2021).

The impact of income inequality and poverty levels on suicide rates across different U.S. demographic groups

The connection between income inequality, poverty, and suicide rates in the U.S. varies across different groups and involves many factors. Poverty is a key factor affecting health outcomes, including suicide rates. It disproportionately impacts marginalised racial and ethnic groups, worsening public health issues and leading to higher suicide rates in these communities (Beech et al., 2021).

Research shows that suicide rates are elevated among certain groups, especially men, non-Hispanic American Indian/Alaska Native populations, and young adults aged 18 to 25 years old (Wang & Wu, 2021). For example, Black males near the poverty line reported a greater risk of suicidal ideation (Wang & Wu, 2021). These findings point to a link between socioeconomic status and mental health outcomes. Higher-income inequality (measured by the Gini coefficient) and poverty rates consistently correlate with increased suicide rates across U.S. states and demographic subgroups (Perry et al., 2022).

Impact of barriers to mental health care for low-income populations and their role in increasing suicide risk

Access to mental health care for low-income people is seriously limited by a variety of barriers that can worsen mental health problems, including the risk of suicide.

When people experience barriers to mental healthcare access, three major implications emerge: (1) untreated mental health issues get worse, (2) severe psychological crises go untreated, and (3) persistent feelings of hopelessness develop (Coombs et al., 2021). Research shows a clear link between unmet mental health needs and higher suicide rates in low-income communities (Coombs et al., 2021). In the

absence of treatment, temporary emotional pain can develop into ongoing suicidal thoughts, while moderate mental stress can become a serious mental illness.

Research gap

Research overlooks how race, wealth, and geography combine to impact suicide risk and care access. Intersectional, community-based studies on economics, rural/Indigenous issues, policy, and tech are needed.

Methodology

This study adopts a systematic literature review technique to objectively analyse three cross-sectional studies from the United States to evaluate socioeconomic impacts on suicide rates and access to mental health care. The carefully developed method comprehensively assesses demographic differences while preserving ethical integrity.

Research Philosophy

Saunders et al. (2018) identify five research philosophies: positivism, interpretivism, pragmatism, postmodernism, and critical realism. Though positivism and interpretivism dominate business research (Sanchez et al., 2022), this study adopts critical realism, which recognises both the objective reality of socioeconomic barriers and the subjective interpretations that influence mental healthcare access and suicide risk.

Research Approach and Research Method

This Research uses two main approaches: deductive and inductive. Deductive reasoning explores known concepts, while inductive reasoning builds a theory from observations (Streefkerk, 2023).

Research Strategy

A research strategy is a systematic plan that directs the finding and assessment of material relevant to a research question. It generally includes many essential procedures to guarantee a thorough and methodical investigation (Cloutier, 2024). This strategy enabled an in-depth exploration of how socioeconomic factors shape suicide rates and access to mental health care in the U.S.

Sample & Sampling Techniques

Purposive sampling will be used to select studies highlighting how socioeconomic factors impact suicide rates and access to mental health care in the U.S. Secondary data ensures comprehensive coverage while acknowledging potential concerns about data quality.

Inclusion and Exclusion of Criteria

Clear criteria were used to select studies relevant to the research aim. The table below outlines the key factors for inclusion and exclusion, focusing on population, healthcare access, quality, and ethical standards in the U.S. context.

Criteria type	Inclusion criteria	Exclusion Criteria
Population	The selected resources are mainly about U.S. residents, with focus on socioeconomic disparities (income, education, race, rural/urban).	Studies not focused on U.S. populations or lacking socioeconomic analysis.
Healthcare Services	Studies on mental health access (insurance, treatment availability, prevention programs).	Research without data on mental health access or suicide prevention.

Quality of Healthcare	Research on treatment effectiveness, barriers (cost, stigma, provider shortages).	Papers that do not address disparities in care quality (e.g., only clinical/biological factors).
Time Period	Studies conducted within the past 10 years (2015+).	Outdated studies (more than 10 years old).
Geographic Scope	U.S. only (national, state, or regional data).	Non-U.S. studies.
Language	English.	Non-English publications.
Ethical Considerations	The selected Sources include research approved by institutional review boards (IRBs) to ensure ethical standards.	The Report will exclude studies with unethical research practices (e.g., non-consented data). It will exclude studies lacking transparency in methodology or funding sources.
Validity And Credibility	Peer-reviewed studies, government reports (NIH), and reputable datasets.	Non-peer-reviewed sources, biased/unverified reports, or studies with small/unrepresentative samples.

Table 2 – Inclusion and Exclusion of Criteria Table.

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PRISMA Diagram

A PRISMA diagram visually outlines the selection process in systematic reviews, helping ensure transparency and clarity (Tricco et al., 2018). For this review, 20 studies were initially identified. After removing 3 duplicates and excluding 7 during screening, 10 full-text articles were assessed. Six were excluded based on design, population, or outcome relevance, leaving four studies included in the final analysis.

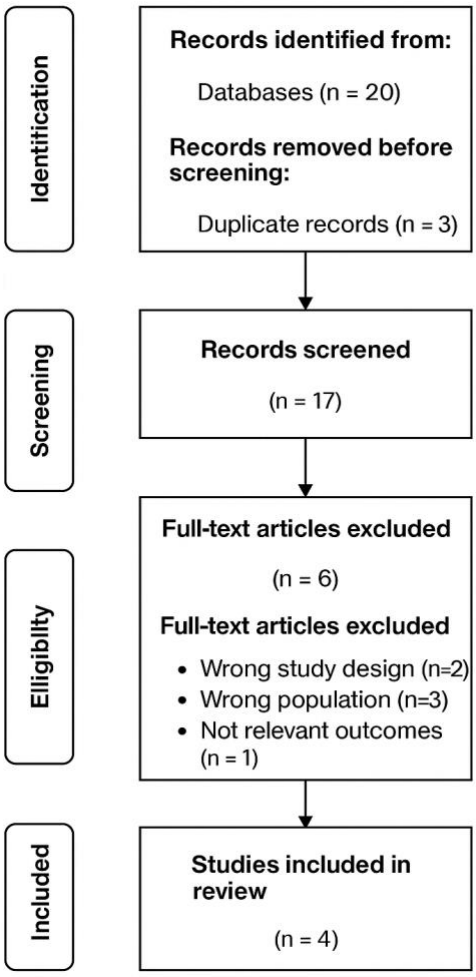


Figure 1 – PRISMA Diagram of the Report.

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Analysis and Discussion

Introduction

This paper conducts a systematic review of the research on the impact of socioeconomic factors on suicide rates and access to mental health care in the United States. Its goal is to discover and comprehend the many elements influencing mental health outcomes and gaps in care accessibility across socioeconomic groups.

Themes Presentation

This part of the Report explores key mental health challenges in the U.S., focusing on common disorders, socioeconomic links to suicide, and how low-income communities face barriers to essential mental health care.

Themes	Title
Theme 1	Prevalent U.S. Mental Health Disorders and Their Impact
Theme 2	Income Inequality, Poverty, and Suicide Rates in U.S. Demographics
Theme 3	Suicide Risk and Mental Health Barriers in Low-Income Groups

Table 3 – Themes Presentation Table.

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Theme 1: Prevalent U.S. Mental Health Disorders and Their Impact

Mental health disorders are prevalent in the U.S., affecting many demographics and significantly diminishing quality of life and overall health. Children show a notable prevalence of treatable disorders, but over 40% of veterans with spinal cord injuries are diagnosed with conditions such as depression and PTSD (McDonald et al., 2017). Mental health challenges frequently correlate with substance use among teenagers and young adults. Active-duty military personnel face greater risks due to combat exposure and deployment experiences (McDonald et al., 2017). According to Golin et al. (2016), women from low-income backgrounds often experience PTSD, significantly influenced by factors like food insecurity and maltreatment. Incarcerated individuals with a history of substance abuse are more susceptible to rule violations, and those with opioid dependence frequently have co-occurring mood or anxiety disorders (Henry, 2020). Children with ADHD and other mental health disorders may exhibit poor responses to treatment (Al Ghriwati et al., 2017). These findings highlight the urgent need for comprehensive, population-specific mental health interventions.

Theme 2: Income Inequality, Poverty, and Suicide Rates in U.S. Demographics

Income inequality and poverty are significantly correlated with suicide rates across various U.S. groups, particularly among the youth. Research indicates that counties with poverty rates over 10% exhibit significantly higher suicide rates, especially those involving weapons (Hoffmann et al., 2020). This trend indicates an ongoing increase in risk corresponding to rising poverty levels. Income inequality at the state level correlates with increased death rates among individuals aged 25–64, showing a 40% rise for men and 14% for women, after accounting for demographic variables (Sorlie et al., 2007). These data highlight that economic differences substantially influence health outcomes, including suicide rates. Suicide rates among youth were elevated in counties identified as having a scarcity of mental health professionals, suggesting that restricted access to care intensifies the impact of poverty (Hoffmann et al., 2022). The research highlights the necessity of addressing socioeconomic disparity and healthcare accessibility to mitigate suicide rates and enhance mental well-being, especially in disadvantaged populations.

Theme 3: Suicide Risk and Mental Health Barriers in Low-Income Groups

Individuals with low income in the U.S. face elevated barriers when accessing mental healthcare, possibly elevating the risk of suicide. According to Coombs et al. (2021), a national study utilising data from over 50,000 participants revealed that those with mental health challenges (MHC), as indicated by levels of psychological distress, had more barriers to care, particularly concerning affordability. More than 60% of those experiencing acute distress expressed concerns regarding the payment of both emergency and routine medical expenses (Coombs et al., 2021). One key barrier examined was not having a usual source of care (NUSC: No Usual Source of Care), meaning no regular place, like a clinic or doctor, to seek help when needed. While MHC alone wasn’t significantly linked to NUSC after adjusting for other factors, affordability concerns were strong indicators. Identifying as male, Hispanic, or lacking access to paid sick leave further increased the probability of NUSC (Coombs et al., 2021). The findings indicate that financial challenges and insufficient workplace support significantly

impede access to care (Coombs et al., 2021). Combating financial and structural impediments is crucial for enhancing mental health outcomes in at-risk communities.

Discussion of Findings

Prevalent U.S. Mental Health Disorders and Their Impact

The findings demonstrate the complex prevalence of mental health issues among U.S. communities. Although prevalence rates are extensively recognised (Hasin et al., 2018; Santo et al., 2021), the interconnection of factors influencing mental health experiences is less frequently addressed. Vulnerable populations, including veterans, imprisoned individuals, and children with ADHD, provide distinct diagnostic and therapeutic issues (McDonald et al., 2017; Al Ghriwati et al., 2017; Henry, 2020). These complex examples emphasise the necessity for specialised public health policies that consider multiple medical conditions, social determinants, and unequal access to care, particularly for women from low-income families whose mental health is influenced by systemic challenges (Golin et al., 2016).

Income Inequality, Poverty, and Suicide Rates in U.S. Demographics

Socioeconomic disparities persistently influence mental health outcomes, with poverty serving as both a direct and contextual risk factor for suicide. The unequal suicide rates among young adults and racially marginalized populations demonstrate the cumulative impact of social inequalities (Beech et al., 2021; Wang & Wu, 2021). Furthermore, regional evaluations indicate that areas with scarce resources encounter increased risks, as economic deprivation and restricted access to providers increase vulnerability (Hoffmann et al., 2020; Hoffmann et al., 2022). These patterns highlight the need for a dual approach addressing income inequality and mental health service gaps.

Structural barriers, particularly affordability and a lack of reliable care sources, are critical variables increasing suicide risk among low-income individuals. Despite experiencing significant psychological discomfort, many individuals avoid therapy due to financial concerns or the absence of a regular healthcare practitioner (Coombs et al., 2021). The data suggest that mental health care policy must go beyond diagnosis and treatment availability; it must also address underlying financial, occupational, and societal constraints that prevent vulnerable populations from receiving timely and adequate support.

Conclusion and Recommendations

This study investigated how socioeconomic factors influence suicide rates and mental healthcare access in the United States, revealing significant disparities shaped by poverty, income inequality, and structural barriers.

Mental health disorders, including Major Depressive Disorder (MDD), anxiety, PTSD, and substance use disorders, are prevalent and disproportionately affect vulnerable populations such as women, young adults, veterans, and imprisoned individuals. These disparities underscore the urgent necessity for tailored mental health strategies that address demographic vulnerabilities.

A significant connection exists between poverty, income disparity, and elevated suicide rates. Individuals living in poverty or areas with evident income inequality, especially young adults, racial minorities, and marginalised groups, experience higher suicide risks. Socioeconomic instability intensifies mental health issues, highlighting the necessity for policies focused on economic fairness and resource distribution to neglected regions.

Structural barriers substantially limit access to mental healthcare for low-income groups. Affordability, absence of insurance, restricted availability of mental health practitioners, and inadequate workplace support impede access to care, elevating suicide risk among economically poor individuals. Mitigating suicide risk necessitates addressing these barriers through enhanced insurance coverage, accessible treatment alternatives, and augmented mental health infrastructure.

Overall, socioeconomic factors significantly influence mental health outcomes and suicide rates in the United States. Combating economic disparity, enhancing healthcare access, and implementing tailored interventions are essential measures for decreasing suicide rates and attaining mental health equity among diverse communities.

Policymakers and practitioners should prioritise targeted measures to reduce suicide risk and address socioeconomic disparities in mental health access. Mandatory comprehensive coverage must be implemented to lower out-of-pocket expenses and improve affordability. Community suicide prevention programs for at-risk groups (like young adults, veterans, and ethnic minorities) should focus on stereotype reduction, education, and cultural communication. Integrating mental health into broader policies (employment protections, paid leave, stable housing) can tackle economic determinants, reduce barriers, and improve mental health equality nationwide.

Reflection

Gibbs' Reflective Model (Bandara, 2023)

Description

This research examined how socioeconomic factors impact suicide rates and access to mental health care in the U.S., using a systematic review of three key studies. The process involved identifying themes, analysing data, and answering research questions related to mental health disparities.

Feelings

Initially, I felt both engaged and challenged. The topic is socially important and emotionally complex, which made me passionate but also cautious. As I became more familiar with the literature, I felt more confident in analysing academic sources and identifying key patterns.

Evaluation

The research process had clear strengths, particularly in its structured approach and strong focus on peer-reviewed evidence. However, one challenge was narrowing down broad themes without losing important details. My lecturer and colleagues' lesson discussions were very helpful in clarifying concepts, strengthening my arguments, and giving me different perspectives on the topic.

Analysis

This project improved my critical thinking, synthesis, and academic writing skills. I better understood how income inequality, poverty, and systemic barriers shape mental health outcomes. It also helped me appreciate the role of policy and infrastructure in healthcare accessibility.

Conclusion

I chose Gibbs' Reflective Model because its structured, step-by-step format supported a

thorough reflection on both the process and outcomes of my work. It helped me evaluate not just what I learned, but how I learned it. Using qualitative and quantitative research provided rich insight, though the absence of primary data limited the personal depth of findings.

Action Plan

In future projects, I will consider mixed-methods research to combine data and personal perspectives. I would also explore government reports, policy briefs, and practitioner insights to complement academic findings and gain a more practical, solutions-focused understanding.

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